

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Propranolol for the treatment of Anxiety (Situational and Somatic Symptoms)

Summary of NICE / NICE CKS Guidelines for Anxiety (Situational and Somatic Symptoms)

Overview

- Anxiety disorders are common, affecting physical and psychological wellbeing
- Physical symptoms include palpitations, tremor, sweating, and gastrointestinal disturbance

Assessment

- Use GAD-7 score to assess severity and impact
- Differentiate generalised anxiety from panic disorder, social anxiety, and PTSD
- Exclude thyroid disease and cardiac causes for physical symptoms

Management

- Cognitive Behavioural Therapy (CBT) is first-line psychological treatment
- Propranolol for symptomatic relief of physical symptoms in situational/performance anxiety
- Propranolol is not first-line for generalised anxiety disorder

Red flags requiring specialist referral

- Suicidal ideation or severe depression
- Severe panic disorder or agoraphobia
- PTSD or trauma-related anxiety
- Comorbid substance misuse

Patient Group Direction

for the supply/administration of

Propranolol 10mg tablets

for the treatment of

Anxiety (Situational and Somatic Symptoms)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC

Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Symptomatic relief of situational/performance anxiety (physical symptoms) in adults
Inclusion criteria	Adults aged 18 years and over Physical symptoms of situational anxiety (palpitations, tremor, sweating) Informed consent
Exclusion criteria	Asthma or history of bronchospasm Uncontrolled heart failure Cardiogenic shock Severe bradycardia (heart rate <50 bpm at rest) Hypotension (systolic BP <90 mmHg) Sick sinus syndrome Second-degree or third-degree heart block Phaeochromocytoma (unless already on alpha-blocker) Metabolic acidosis Prinzmetal's angina Concurrent verapamil or diltiazem, oral or intravenous. With a beta-blocker the combination risks severe bradycardia, heart block and hypotension. Known hypersensitivity to propranolol Suicidal ideation, self-harm, or severe depression. Propranolol is cardiotoxic in overdose. Refer. PTSD, severe panic disorder or agoraphobia. Refer. This PGD is for the

	physical symptoms of situational anxiety only. Comorbid substance or alcohol misuse. Refer. Already taking another beta-blocker.
Cautions	Diabetes (masks hypoglycaemia symptoms) First-degree heart block Portal hypertension Peripheral vascular disease Psoriasis Raynaud's Myasthenia gravis History of anaphylaxis Depression that is not severe and without any suicidal ideation. Supply may proceed with counselling. Severe depression or any suicidal ideation is an exclusion, above. Hepatic/renal impairment Elderly Do not stop abruptly
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Propranolol 10mg tablets
Legal category	POM
Storage	Below 25°C, protect from light
Route/method of administration	Oral
Dose and frequency	10-40mg taken 30-60 minutes before anxiety-provoking situation Alternatively, 10-40mg two to three times daily for ongoing situational anxiety Maximum 120mg daily
Quantity to be administered and/or supplied	Up to 28 tablets of 10mg, 280mg of propranolol in total, and no more. Propranolol is cardiotoxic in overdose and the whole supply taken at once must remain below 320mg. One supply per situational event or course. Review before any repeat. The 40mg strength is not authorised under this PGD.

Maximum or minimum treatment period	Review at 4 weeks; ongoing use should be reviewed regularly. Consider gradual dose reduction if discontinuing.
Adverse effects	Common: fatigue, cold extremities, bradycardia, GI disturbance, sleep disturbance, dizziness Uncommon: bronchospasm, hypotension, heart failure exacerbation Rare: alopecia, visual disturbance, psoriasiform rash

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Do not stop suddenly, report breathlessness/wheeze, may cause cold hands/feet, not a cure for anxiety - consider psychological therapy, avoid alcohol (additive CNS depression)

Key references

• NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 • NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
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<https://www.nice.org.uk/guidance/mpg2/resources>

- NICE CKS Guidance: Anxiety disorders
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document, and any correction notice attached to it.

Change: Version 001 authorised a 40mg arm and printed the same quantity line for both strengths, up to 56 tablets, which at 40mg is 2,240mg of propranolol. Propranolol is cardiotoxic in overdose. The 40mg arm is withdrawn, and the supply is capped at 28 tablets of 10mg, 280mg in total, so that the entire supply taken at once remains below 320mg. The document's own guidance named suicidal ideation, severe depression, PTSD and substance misuse as red flags and none of them was an exclusion. They are now, with another beta-blocker, and the verapamil and diltiazem exclusion covers the oral forms as well as intravenous. This incorporates and withdraws the correction notice of 7 September 2026.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.